My Record Against the 100% Criteria

An honest measure of where the file supports, contradicts, or is silent on a total rating

2026-10-13

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1. Bottom line

Where the record stands. Read as a whole, the record supports a 70% mental-health rating well and a 100% rating poorly.

- **Work is severe.** Treatment or leave interrupted every documented job from 2022 to 2026, and I haven't worked since June 2026. Two points are still open: the July 2023 overlap between Meta and Starlite, and how DISQO ended.
- **Social life is badly impaired, but not "total."** The marriage continues.
- **Self-care is mixed.** Erica describes both reminding me and doing things for me: making my appointments, replacing worn clothes, grocery shopping and managing my sleep medications. My own 10/8 account describes prompting.
- **The closest the record comes to the 100% list is in Cherie's letter.** She writes that during "especially bad stretches, he is unable to keep up with basic self-care, and his wife takes over for him," and she uses the phrase "paranoid delusions" once.
- **No examining clinician's mental status exam documents any of the seven 100%-level example symptoms.** Naficy recorded me as oriented, memory intact, and with no delusions or hallucinations at five visits. The two ER visits recorded me as oriented.

The strongest route to being paid at the 100% rate is not a schedular 100%. It is a 70% grant plus TDIU under 38 CFR 4.16(a), and that requires service connection first. A single disability rated 70% clears the 60% threshold.

The biggest obstacle comes in two layers:

- **Service connection,** which turns on credibility: the disclosure timeline, the five earlier trauma denials and the pre-service history.
- **For any total rating,** a record that reads as episodic. That means:
  - the April–May SI denials;
  - the "well-groomed" entries;
  - the 8/4 hospital record, which calls that overdose accidental and records an SI denial;
  - the 8/5 ER record, which codes "ACUTE SUBSTANCE ABUSE" with a "Low risk" nursing screen;
  - a growing set of employability facts: recruiter offers, consulting plans, and possible VR&E and unemployment statements.

The three things that would most change the picture:

1. **What the C&P examiner found.** The overdose 21-4138 is filed, and its text is now in `personal-records/21-4138-AUGUST-2026-OVERDOSES-FILED.md`. It gives the reasons for the hospital account. It doesn't address the other medications the 8/4 record documents (§4 row 1).

2. **A longitudinal treating record covering the time since August.** That means Cherie's full chart from 2/2023, any post-August notes, and possibly a treating-provider DBQ or independent evaluation in each clinician's own judgment. A record like that would show whether my course is persistent or episodic.
3. **The truth about work capacity, settled and consistent across every VA-visible file.** That covers the 9/17 VR&E outcome, any Texas unemployment certifications, and the consulting plans. TDIU rises or falls on this.

2. What 100% actually requires

The regulation. 38 CFR 4.130 sets six levels for mental disorders: 0, 10, 30, 50, 70 and 100%. **There is no 90% level** (`rater-kb/criteria-4130-caselaw.md` §1). All of my mental diagnoses (PTSD, MDD, GAD, OCD, and SUD if it's found secondary) get one combined rating. Any plan built on a "90% mental health" step, including the stale `REPORT.md` "Start here" route, doesn't work.

The 100% level reads: "Total occupational and social impairment, due to such symptoms as: gross impairment in thought processes or communication; persistent delusions or hallucinations; grossly inappropriate behavior; persistent danger of hurting self or others; intermittent inability to perform activities of daily living (including maintenance of minimal personal hygiene); disorientation to time or place; memory loss for names of close relatives, own occupation, or own name" (38 CFR 4.130; `criteria-4130-caselaw.md` §1).

The 70% level reads "deficiencies in most areas, such as work, school, family relations, judgment, thinking, or mood." Its examples include suicidal ideation, obsessional rituals, near-continuous panic or depression, neglect of personal appearance and hygiene, difficulty adapting to stress, and inability to establish and maintain effective relationships (same source).

In plain language:

- **"Most" versus "total."** At 70%, problems span most areas of life. At 100%, impairment is total in **both** work **and** social life at the same time. Being severely impaired at work while keeping some working relationships doesn't meet 100% on that basis alone (`criteria-4130-caselaw.md` §2). VA also may not rate on social impairment alone (38 CFR 4.126(b)).
- **The symptom list is examples, not a checklist.** *Mauerhan v. Principi*, 16 Vet. App. 436 (2002), holds that "such as" makes the listed symptoms illustrations of the type and degree of impairment. A symptom that isn't named can still count.
- **But symptoms must cause the level.** *Vazquez-Claudio v. Shinseki*, 713 F.3d 112 (Fed. Cir. 2013), requires the listed symptoms "or others of similar severity, frequency, and duration," and they must produce the impairment. Read together, the two cases mean neither the words on the list nor a count of symptoms decides the rating. Function does.
- **Suicidal ideation.** *Bankhead v. Shulkin*, 29 Vet. App. 10 (2017), holds that SI, including passive SI with no plan or intent, "may" support 70%. It doesn't automatically compel 70%, and it doesn't relax the "total" standard for 100% (`criteria-4130-caselaw.md` §3.3). The 10/8 path document reports that Bankhead also

treats ideation (a 70% symptom) as distinct from "persistent danger of hurting self" (a 100% symptom), and that treating them as one is error. That was citation-verified against the slip opinion (`PATH-TO-100-WITHOUT-TDIU-2026-10-08.md` §3, §7).

- **The whole record counts, not just exam day.** VA weighs "the frequency, severity, and duration of psychiatric symptoms, the length of remissions, and the veteran's capacity for adjustment during periods of remission" (38 CFR 4.126(a)). That cuts both ways for me: bad stretches count, and so do the stretches between them.
- **How the examiner records it.** The DBQ has one "check only one" box summarizing impairment across all mental diagnoses. The top box is "Total occupational and social impairment" (`rater-kb/dbq-mental-disorders.md` §1.1). If symptoms can't be separated between diagnoses, impairment is treated as one whole (§1.2).

3. Domain-by-domain scorecard

Verdicts: **Meets** (the 100% standard), **Approaches** (clearly past 70%, short of 100%), **Doesn't meet** (at or below 70%), or **Silent**.

3.1 Occupational: the first axis

Standard	Record supports	Record contradicts	Verdict
100%: total impairment, meaning unable to work in any setting. **70%:** deficiencies in work; difficulty adapting to stress, including work.	Unemployed since June 2026. MNTN "terminated" me effective 6/26/2026, with severance and no reason stated (`personal-records/MNTN-Separation-Agreement-2026.pdf` opening ¶). My 10/7 21-4138 says the job ended "related to my disabilities, after a period of FMLA leave beginning April 17, 2026" (Part 2 item 5). The Metal manager wrote that I called my condition "debilitating" on 7/18/2024, said the days I worked weren't productive, and had sick leave on 7/18, 11/15 and 11/21/2024 (`MetalManagerEmploymentStatement.pdf`). That statement ties the 11/15 episode to "an interruption in medication," which an examiner may attribute to medication rather than to PTSD. Treatment or leave hit every documented job 2022–2026 (the July 2023 Meta/Starlite overlap and how DISQO ended are unresolved): Starlite stays in 1/2023, 7–8/2023, 7–8/2024 and 12/2024; Better Days 9/2025; Harmony 4–5/2026 (`CP-EXAM-PREP-2026-10-13.md` §3). WHODAS self-report: totally unable to work 10 of the past 30 days (6/11/2026). Cherie: "severe to total impairment in the occupational realm"; "I have witnessed him pathologically overwork, ultimately resulting in a collapse into unemployment" (7/17 letter).	Meta ended in a WARN-notice layoff (`eb99f8e9-…-signed.pdf`). Metal ended in a "headcount reduction for financial reasons" (Metal statement). Same-period records: Starlite 8/2024 "gainfully employed"; Victory 2018 "employed," "good work ethics" (Victory pp. 7, 37). Recruiter offers of $195–229K as late as 9/12/2026, an AWS certification, a 1/2025 CTO offer, and consulting-practice plans as of 9/14 (`vre/OPTIONS-REPORT-2026-09-14.md` §1, §3; `vre/VRE-EVALUATION-PREP-2026-09-17.md` §3.2, §5.4). Cherie's letter, which reads overwork as a symptom ("not evidence of capacity"), also records that I "pathologically overwork" and was "obsessively consuming himself in work," and refers to evidence "regarding his value in the role." A rater may read those passages as capacity.	**Approaches.** Severe, but the record shows jobs obtained and lost, not total inability.

How an RVSR is likely to read it. The pattern is real: I can get senior jobs, and I can't keep working steadily through them. That is the strongest part of my file, and it fits the 70% language ("difficulty in adapting to stressful circumstances (including work or a worklike setting)") better than "total." Two of four separations were company layoffs. A credibility reviewer will set "my disabilities ended my job" against those records (`rater-kb/denial-patterns.md` §2, income/work-history conflicts). The cleanest occupational facts are:

- the MNTN termination after FMLA leave;
- the Metal attendance record;
- the treatment-interruption timeline.

Each holds up on its own documents.

3.2 Social and family: the second axis

Standard	Record supports	Record contradicts	Verdict
100%: total social impairment, with no functioning relationships. **70%:** deficiencies in family relations; "inability to establish and maintain effective relationships."	Erica: "for years he has had no social life at all"; I stopped speaking to friends; she avoids bringing me into social situations; aversion to most women (7/12 statement). Cherie: outside my wife I regularly speak only to her; I left a crowded 12-step "birthday night" on arrival (7/17 letter). Family: strained, little contact (veteran's account, 10/8–10/13). On guard around women at gatherings, expecting harm, and moving away (veteran's account, 10/13). Cherie's progress notes check "social isolation" and "lack of social support."	The personal statement says "minimal social engagement **outside my immediate family**" (§8). Erica describes planning "a move back to his hometown," and says I felt that meant "moving back to where his family is." The `vre/` documents place me in Lake Charles from about 9/2026 (`vre/VRE-EVALUATION-PREP-2026-09-17.md` §3.1 row K, marked "inferred"; `vre/FINANCIAL-BRIDGE-2026-09.md` header). That is family proximity, which sits alongside "little contact." Cherie's 5/13 note mentions "boundaries… with his family" (`PATH-TO-100-WITHOUT-TDIU` corrections). AA attendance and about one outing a week since the last stay (Cherie). Poquette: I "reached out" and "stayed in touch here and there." 8/4 ED: "Adequate social support at home." Cherie's "estranged… for the past 6 to 7 years" goes past my own account.	**Approaches.** Severely limited, not "total."

3.3 Marriage

The marriage is the most important single social fact, and the record describes it three ways:

Source	What it says
My signed personal statement (6/17/2026)	Intimacy problems "even though we have a healthy and supportive relationship" (§8). The occupational paragraph of the same section says "My marriage to Erica is strained by the symptoms above" (§8).
Erica (7/12/2026)	"Roommates at best and enemies at worst." Verbal cruelty about her income. She "seriously considered divorce at several points." "A couple of months apart." She stays out of hope that it can be repaired.
Cherie (7/17/2026)	"An area of total impairment… a caregiving dependency relationship," with "no reciprocal companionship over the last 5 to 6 years."

Verdict: Approaches. The marriage is badly strained and partly caretaking, but it continues. The Board decisions sampled in the corpus show that a marriage that continues doesn't by itself defeat 100%. Grants exist where the veteran lives with a spouse but isolates within the marriage, and denials cite a "good" or "supportive" marriage (`PATH-TO-100-WITHOUT-TDIU` App. A; non-precedential, non-random sample). My signed statement contains both "healthy and supportive" and "strained." Only I can say whether each was accurate when I wrote it (§9).

3.4 Self-care, ADLs and competency

Standard	Record supports	Record contradicts	Verdict
100%: intermittent **inability** to perform ADLs, including minimal hygiene. **70%:** neglect of personal appearance and hygiene.	**Cherie (7/17 letter):** "During especially bad stretches, he is unable to keep up with basic self-care, and his wife takes over for him. She washes and picks out the clothes for him to wear, needs to remind him to brush his teeth…" She also writes that I've come to sessions with "minimal personal hygiene, disheveled clothing and grooming," and that adequate grooming came with my wife's help. **Erica (7/12), reminding:** "During the worst bouts… I would find myself having to constantly remind him to brush his teeth and shower." **Erica, doing:** "I have to make his appointments for him"; "I would have to throw away clothes that were full of holes and buy him new ones"; "I would dust crumbs and dandruff from his face and clothes"; "I would grocery shop for him"; "I had to start managing his sleep medications." **Veteran's account, 10/8:** Erica prompts and manages haircuts, shaving, showering, eating, water, laundry and doctors.	**Naficy, at five visits 4/18–5/5/2026:** "well-groomed, clean"; oriented; memory intact (`marcus 3 (1).pdf` and the other Naficy notes). **Victory 2018:** "Performs own ADLs." **Cherie:** hygiene "improved" during FMLA leave. I've been off work since June, so her own reasoning points to improvement now. Neither Cherie nor Erica gives a frequency for the "bad stretches" or "worst bouts."	**Approaches.** Cherie's "unable… takes over" sentence is a treating clinician's statement close to the 100% wording ("intermittent inability"). Erica's account adds things she does for me, not just reminders. Against that are five examining-clinician MSEs recording good grooming, the absence of any frequency, and Cherie's own FMLA-improvement point. In the Board sample, family-reported ADL help that no examiner documented was found insufficient (A26005755, A25108231; `PATH-TO-100-WITHOUT-TDIU` App. A).

Competency. Three spending figures exist and don't reconcile:

- more than $30,000 on 7-OH over about two years (veteran's account, 10/8);
- about $500 a day at the time of the overdoses (veteran's account, 10/8);
- "around $30,000 on drugs over the course of our marriage" (Erica, 7/12).

The first two are mine to reconcile (`CP-EXAM-PREP-2026-10-13.md` §9 item 12). Erica's figure is her own knowledge and stays as she wrote it. Erica also manages my sleep medications. The record is **silent** on who handles the money now.

3.5 Safety: SI, overdoses, danger to self or others

Standard	Record supports	Record contradicts	Verdict
100%: persistent danger of hurting self or others. **70%:** suicidal ideation (passive counts, per *Bankhead*).	**SI:** the personal statement says SI "hits every day when I wake up," with no plan and never acted on. Cherie: "suicidal thoughts daily," from passive wishes to "more intense thoughts of death," no plan or intent. Erica: "afraid to leave him alone out of fear he will harm himself." **The overdoses:** two ER visits, on 8/4 and 8/5/2026. My account (veteran's account, 10/8–10/13; draft at `21-4138-AUGUST-2026-OVERDOSES-DRAFT.md`): both were 7-OH at toxic doses, with SI at the time, and I understated both. **Contemporaneous support for 7-OH use:** the 8/5 record names kratom; Naficy 4/18 records "7OH use 2 years… Last use 4/17/26," repeated on 5/5. **Danger to others or self:** hostile and abusive toward Erica at the overdoses (draft item 3). Erica describes two driving passages. First, I couldn't stop answering work messages while driving, which led to "four accidents in our apartment parking garage"; once, driving and texting with the dog in the car, "his driving was so erratic he appeared to be driving drunk even though he was sober at that time." Second, she describes me taking sedating sleep medications at odd hours. She has seen me "sleepwalk and drive his car in this inebriated condition," and she writes that I put "himself, myself, other people, and our dog in danger by driving under the influence or while running on no sleep."	**8/4 (South Austin Medical Center):** "accidental overdose of 1200 mg of gabapentin." "He denies any suicidal ideation or thoughts of self-harm." "He states he was not trying to hurt himself." Tele-psych: "He's not a danger to himself at this time. No SI. No HI." The same record also shows propranolol, Valium, clonidine and dimenhydrinate; a positive benzodiazepine screen and a negative opiate screen; and impressions of antihistamine poisoning, gabapentin overdose and "Polypharmacy." Admitted at 06:04, discharged home at 6:35 PM the same day (`08042026_Overdose_Discharge.pdf`, `_Physical.pdf`, `_Provider_Report.pdf`, `_Summary.html`). **8/5 (ER):** "KRATOM OD"; impression "ACUTE SUBSTANCE ABUSE"; nursing "Calculated Suicide Risk… Low risk," which is a screen result, not a recorded denial; discharged from the ER (`08052026_Overdose_ER.pdf`). **Spring:** seven SI denials, 4/18–5/14/2026 (Naficy ×5, the Harmony discharge, the FMLA evaluation). Cherie's "high risk… **if treatment discontinues**" is a conditional template line repeated in her progress notes, not a current-danger finding.	**Doesn't meet "persistent."** SI meets the 70% example.

How it reads. Daily SI is the clearest 70%-level fact in the file. "Persistent danger" is a different and higher thing (*Bankhead*, as verified in the corpus). Today the record shows two ER visits, on 8/4 and 8/5. The first is coded accidental, with an SI denial and a tele-psych "not a danger" finding. The second is coded acute substance abuse, with a low-risk screen. The spring SI denials sit around them. In the Board sample, an isolated attempt or overdose followed by stabilization kept the rating at 70%, and persistent or escalating suicidality won 100% (`PATH-TO-100-WITHOUT-TDIU` §3 and App. A). The overdose 21-4138 adds my account of SI at the time. It is a later self-report that contradicts the 8/4 record, and VA will weigh it that way. The 8/5 kratom entry and the Naficy 7-OH history give contemporaneous support for the substance part of that account. The 8/4 record's other medications and negative opiate screen don't contradict 7-OH use (no kratom test was run, per `CP-EXAM-NOTES-2026-10-13-SOURCES.md` §4 item 2), but they do show the polypharmacy a reviewer will also see.

3.6 Thought process and content

Record supports:

- Cherie's letter lists "paranoia" among my symptoms. It uses "paranoid delusions" once, as a separate item in the phrase "the stress, paranoid delusions, and constant fear of losing his job."

- Erica describes a “near constant deluge of paranoia,” rants about society collapsing, and ranting that “becomes tangential and ultimately doesn’t fit the reality.”
- Cherie’s progress notes record “Obsessive” and “Pressured Speech.” The 5/13 note adds “Impaired Judgment.” That note is coded F33.1 (recurrent MDD) and was created and signed on 6/25/2026, a late entry (`Cherie CPT Progress Note 5.pdf`).
- My 10/13 account: a certainty that I’ll fail despite results. At McNeese I was sure I’d fail tests and got A’s; I’m sure every project will fail; I’m sure people won’t like me.
- My settled account (10/8–10/13): a recurring persecutory worry.

Record contradicts:

- Naficy: “Thought Processes: intact”; “no hallucinations”; 4/18 “denies… delusions.”
- Harmony: “denied… psychotic symptoms.”
- My settled account (10/8–10/13): the persecutory belief is a **recurring worry, not a fixed delusion**, and “grandiose” was ordinary self-belief.

Verdict: Doesn’t meet. Cherie’s one use of “paranoid delusions” stands against five examining MSEs with no delusions and against my own account. No source documents persistent delusions or gross impairment of thought. The anxious, catastrophic thinking is real and belongs in the record at its true frequency and conviction. It sits at the 50–70% level.

3.7 Memory and orientation

- **Supports:** I forget appointments, and Erica schedules and tracks them (Cherie, Erica).
- **Contradicts:**
  - Naficy, at five visits in 2026: oriented, with recent and remote memory normal.
  - Victory, 2018 discharge: “oriented in all spheres,” with good memory recall.
  - The 8/4 and 8/5 ERs recorded orientation only. On 8/4 I arrived “awake drowsy but answering questions,” with horizontal nystagmus.
  - Cherie’s progress notes: “Alert and Oriented.”
- **Verdict: Doesn’t meet.** No source records memory loss for close relatives’ names, my own occupation or my own name, or disorientation to time or place.

3.8 Behavior

- **Supports:**
  - Erica: meltdowns over menu prices; anger when hygiene is raised; lying about being in detox; staying up all night gaming and taking sedatives at odd hours.
  - Personal statement: leaving Zoe PHP early over coed housing.
  - Cherie’s progress notes: tearful and “Emotionally Dysregulated” on 7/3; “Restlessness,” “Fidgety.”
- **Contradicts:** “cooperative, calm, pleasant” (Naficy).
- **Verdict: Doesn’t meet** “grossly inappropriate behavior.” It fits 50–70% irritability and dysregulation.

3.9 Substance use

Severity:

- Harmony 2026 discharge: opioid, sedative and amphetamine-type use disorders, with "polysubstance use" (`marcus DC summary (1).pdf`).
- Treatment in 2016–2018, then roughly yearly 2022–2026.
- About two years of heavy 7-OH (veteran's account; Naficy 4/18).
- An early-June 2026 relapse involving opioid pain pills and kratom, with Suboxone stopped (`COMPREHENSIVE-FINDINGS-2026-07.md` §2; `vre/VRE-EVALUATION-PREP-2026-09-17.md` §3.1 J2).
- The August ER visits.

Attribution: whether this counts toward the rating depends on SUD being found secondary to PTSD (§5). Cherie gives an "at least as likely as not" secondary opinion in her letter body. Her formal Conclusion paragraph names only PTSD, MDD, OCD and GAD (7/17 letter).

Open:

- Cherie's 7/9 and 7/16 notes say "history, currently sober." That conflicts with the early-June relapse and with my account of heavy 7-OH use before August (`CP-EXAM-PREP-2026-10-13.md` §4 item 8).
- Current use is **undated and unquantified, not blank.** The signed 10/9 correction says "my struggles with it continue to this day," and the 10/8 draft says "I still struggle with using it."

3.10 Summary

Domain	Verdict
Occupational	Approaches
Social / family	Approaches
Marriage	Approaches
Self-care / ADLs	Approaches (one treating-clinician statement near 100%; examiners record good grooming)
Safety	Doesn't meet "persistent" (SI fits 70%)
Thought process/content	Doesn't meet
Memory / orientation	Doesn't meet
Behavior	Doesn't meet
Substance use	Severe; counts only if found secondary

Read under *Vazquez-Claudio*, this is a strong 70% picture: deficiencies in work, family relations, mood and judgment, plus SI. Self-care comes closest to the 100% wording, because of Cherie's "unable… takes over" sentence and Erica's account of doing things for me. That still rests on one treating clinician's statement with no frequency, against five examining MSEs. The picture isn't yet a total one.

4. Where I'm likely under-reporting

The aim is completeness, not amplification. Each item lists the evidence that something is understated, what the full truth would add, and an honest way to get it into the record. Some items run the other way: a document of mine overstates, and the fix is the same, which is to put the accurate version in.

#	What	Evidence I understated it	What the full truth adds	Honest way in
1	**August overdoses: cause, amount, SI**	**8/4:** "accidental" gabapentin overdose, "denies any suicidal ideation," "not trying to hurt himself." **8/5:** "KRATOM OD," "ACUTE SUBSTANCE ABUSE," a "Low risk" nursing screen. My settled account (veteran's account, 10/8–10/13): both were 7-OH at toxic doses, with SI.	The real substance, the real amounts, my state of mind, and why I told staff otherwise.	**Filed** (`personal-records/21-4138-AUGUST-2026-OVERDOSES-FILED.md`). It gives two reasons: I blamed gabapentin to hide opioid use from Erica, and I denied SI to avoid being kept or committed. The second matches Cherie's hold-avoidance explanation. It also says there was no intent to die ("did not care whether I died… wanted to escape"). **Open:** it says gabapentin "is not accurate" but doesn't address the propranolol, Valium, clonidine and dimenhydrinate, the positive benzodiazepine screen, or "polypharmacy" in the 8/4 record.
2	**SI denials at treatment admissions**	Cherie's letter: admission denials were "an attempt to control the situation and avoid being placed on involuntary hold." The 07-07 planning account goes further: deliberate minimization across about nine treatment episodes. It appears in **no signed filing** (`PATH-TO-100-PERCENT-2026-07-07.md` I-2(d); stale history).	Why the charts read "denies." **Limit:** mid-May 2026 was genuinely SI-free (my 10/8 account), so the explanation can't be applied across the board.	Only where true, entry by entry: my own 21-4138; Cherie in her own words; or an independent evaluator who reviews the charts. A blanket statement would be inaccurate for May.
3	**Trauma denials to clinicians** (2015 VA, Victory 2018 ×2, Starlite)	"Denies MST," PC-PTSD 0/4, "denies at this time," "None reported"; three years with Cherie before telling her on 6/17/2026.	The avoidance and shame behind each one. *AZ & AY v. Shinseki* protects non-reporting **in service**, not these later denials, so they get weighed for credibility (`CP-EXAM-PREP-2026-10-13.md` §4 item 2).	The exam itself, or an independent psychologist's opinion addressing the disclosure timeline (`REASSESSMENT-2026-10-08-CP-EXAM.md` §3).
4	**Marriage**	My signed statement has both "healthy and supportive" and "strained by the symptoms above" (personal statement §8). Erica and Cherie describe separation, divorce considered and caretaking.	Whichever is true. Only I can say whether "healthy and supportive" was accurate when I wrote it.	The exam itself, if asked. Erica's statement already speaks for her; nothing should be asked of her. Listed as a conflict in §9.
5	**Source conflict: "well-groomed" entries**	Not evidence of my under-reporting. Naficy's "well-groomed, clean" ×5 is a clinician's observation. It conflicts with Cherie's "minimal personal hygiene" in session and with Erica's account.	How often self-care lapses, and what Erica does. The record is **silent** on frequency.	The exam, with real examples. Cherie's and Erica's observations are theirs to record.

#	What	Evidence I understated it	What the full truth adds	Honest way in
6	**Substance use not told to Cherie**	7/9 and 7/16: "history, currently sober," against the early-June 2026 relapse (opioid pills and kratom, Suboxone stopped; `COMPREHENSIVE-FINDINGS-2026-07.md` §2) and two years of 7-OH. The 10/9 checklist asks me to tell her.	An accurate treating record; it also explains part of the SI and August picture. Current use is undated and unquantified ("my struggles with it continue to this day," 10/9 correction).	Tell Cherie. Whether and how she documents it is her decision.
7	**Pre-service history**	Personal statement: "minor alcohol and maybe a little marijuana." 10/7: none besides marijuana. 10/9: kratom from 18, used "infrequently." Victory p. 8: opiate pills, regular use 18–20, and cocaine at 19. The 10/9 correction addresses both: it says I believe the opioid entry reflects kratom and that I don't recall cocaine before service. Starlite intake: use from about 18. Intake self-reports (OCR) also record an age-18 DUI, later expunged, and an age-20 paraphernalia arrest (`RATING-REPORT-REMEDIATION-PLAN-2026-07-16.md` Task 8).	Consistency. It's the most checkable credibility gap in the file (`REASSESSMENT` evening update).	If the 10/9 "infrequent" doesn't match my memory of ages 18–20, or the arrest entries are accurate, a correcting 21-4138 is the clean fix. James should advise.
8	**Omitted treatment stay**	10/7 Part 2 leaves out the Starlite stay of 7/30–8/13/2024 and dates another stay "February 2023."	A complete timeline. The 2024 stay falls during Metal and fits the manager's 7/18 "debilitating" note.	A clarifying 21-4138 (`CP-EXAM-PREP-2026-10-13.md` §9 item 10).
9	**Symptoms with no frequency recorded anywhere**	OCD rituals (no specific ritual is described anywhere), panic frequency, hours of sleep, worry conviction, the women-at-gatherings pattern, the certainty-of-failure pattern (10/13), current use.	The frequency, severity and duration evidence that *Vazquez-Claudio* requires. Without it, an examiner can't map these symptoms to a level (`denial-patterns.md` §2).	The exam, or a post-exam 21-4138 if they weren't covered (`CP-EXAM-PREP-2026-10-13.md` §8 item 2).
10	**August events missing from the VR&E file**	No `vre/` document mentions the overdoses, the hospitalizations or SI, though all were written after August, and VR&E shares my C-file (`vre/` grep; Slice C §3 U1). Sobriety is listed as "UNKNOWN."	The 28-1902w substance and treatment sections may be incomplete.	If the 28-1902w left out the August stays, correct it with the VRC. Accurate cross-file records protect both claims.

Overstatements to correct, not use. Several of my or my supporters' documents run hotter than my own settled account:

- "Estranged… for the past 6 to 7 years" (Cherie), where my account is strained, with little contact.
- "FMLA… at every job he has had since I have worked with him" (Cherie), where Metal (2024) was sick leave and accommodations, not FMLA. My own account lists FMLA at Meta, DISQO and Mountain.
- "Paranoid delusions" (Cherie, once), where my account is a recurring worry and certainty of failure.

- "Feeds him, cuts his hair" (`RATING-REPORT-REMEDIATION-PLAN-2026-07-16.md` [CORRECTION 3]). My 10/8 account is that Erica prompts haircuts and eating. Erica's own statement mentions neither feeding nor haircuts.
- "Fixed belief… nuclear bunker" (07-06 docs), where my account is a recurring worry.
- The 10/9 "after 2024" date error (I separated in 2013). It was corrected, per my account; the fix isn't in the corpus.

An examiner who finds the gap between these and my own account will discount both. Only Cherie can change Cherie's words. Mine I can correct.

5. What weighs against a total rating

This is the record as an RVSR or a hostile reviewer would read it.

1. **Employment and achievement.**
   - Senior engineering jobs at Meta, DISQO, Metal and MNTN from 2022 to 2026.
   - B.S. in Computer Science in 2016, completed during active addiction; A's at McNeese.
   - Recruiter offers of $195–229K as late as 9/12/2026; an AWS certification (inferred from a recruiter email); a remote CTO offer in 1/2025 (Gmail and corpus, unverified; `vre/VRE-EVALUATION-PREP-2026-09-17.md` §3.2, §5.4).
   - Two layoffs among four separations.
   - The WHODAS and DASS-21 were taken while I was employed, through an FMLA vendor.
2. **Statements about capacity that VA may see.**
   - The VR&E prep suggested a framing: "I can work full time… What I haven't been able to do is sustain it." It's unknown whether I said it at the 9/17 evaluation (`VRE-EVALUATION-PREP` §8.3, §9.0.2).
   - The 9/17 VRC notes and any entitlement finding are now C-file records. The corpus doesn't have them.
   - A 9/14 career preference for a solo AI consulting practice (`OPTIONS-REPORT` §1).
   - A possible Texas unemployment claim, which requires weekly certification that I'm "able" to work. Whether it was filed is unknown (`vre/FINANCIAL-BRIDGE-2026-09.md` §1.8).
   - The family CPA-firm move, flagged in the corpus as an optic risk and confirmed on 10/8 as having "gone nowhere."
   - If any of these say I can work, they sit directly against "total occupational impairment" and against TDIU. What's true has to be the same in every file.
3. **"Well-groomed" and "denies" entries.** Naficy's five MSEs (well-groomed, oriented, memory intact, thought intact, no delusions) and seven SI denials across April–May. "Insight: intact" and "poor insight" appear in the same notes, which a reviewer may read as template charting either way.
4. **SI inconsistency.** "Every day when I wake up" (6/17) against seven denials weeks earlier. My own account is that SI fluctuates, which fits *Bankhead* at 70% but not "persistent danger."

5. **The August records themselves.** The 8/4 record calls that overdose accidental, records an SI denial and a tele-psych "not a danger to himself" finding, and shows a same-day discharge. The 8/5 record codes acute substance abuse, with a "Low risk" screen, and discharge from the ER. My later 21-4138 contradicts the 8/4 record, after the claim was filed, and that makes it a credibility question in both directions.
6. **SUD attribution.**
   - Secondary SUD is compensable (*Allen v. Principi*, 237 F.3d 1368 (Fed. Cir. 2001); 38 CFR 3.301(c)(3)). Primary drug abuse isn't.
   - If symptoms can't be separated, they are attributed to the service-connected condition (*Mittleider v. West*, 11 Vet. App. 181, 182 (1998)).
   - The risk runs the other way here. If an examiner separates the two and finds substance use primary, which pre-service kratom and opioid entries invite, much of the occupational evidence becomes SUD treatment and drops out. The corpus estimates roughly half (`CP-EXAM-PREP-MENTAL-HEALTH.md`).
   - The PTSD DBQ tells examiners not to mark symptoms "clearly not attributable" to the stressor (`dbq-mental-disorders.md` §2.2).
7. **Mountain's performance framing.** The agreement gives no reason. My personal statement mentions a "threat of a Performance Improvement Plan," and the remediation plan describes it as a performance/PIP loss. A rater can read "performance" as either impairment or ordinary job loss.
8. **Presentation against the longitudinal record.** An examiner weighs what they observe in the interview together with the whole record. VA must consider "the frequency, severity, and duration of psychiatric symptoms, the length of remissions, and the veteran's capacity for adjustment during periods of remission" (38 CFR 4.126(a)). The exam is one observation. The five Naficy MSEs and the August records are others.
9. **Credibility overhang.** A rater will see:
   - the disclosure timeline, with Cherie told on 6/17/2026, four weeks after the 5/19 claim;
   - the perpetrator described as a "female service member" on the 21-0781, since corrected;
   - two nexus letters;
   - nexus-letter provenance questions;
   - the latent coached-email exposure (`CLAIM-ASSESSMENT.md` Part 0).

   None of these bars a rating, but each lowers the weight of my own descriptions.

6. Other routes to a total rating

6.1 Combined-rating math and the ankle conflict

My signed personal statement says the left ankle is "now service connected at 10% under DC 5271, effective May 19, 2026" (§6). Every other document lists tinnitus at 10% as the only service-connected condition. An effective date equal to the pending claim's receipt date couldn't have been granted by 6/17 (`PATH-TO-100-WITHOUT-TDIU` corrections). **This is unresolved;** James should check the rating code sheet. Here is how the math changes (38 CFR 4.25, combined values rounded to the nearest 10):

Mental-health outcome	Tinnitus only	Tinnitus + ankle 10%	Meets 4.16(a)?
100%	100%	100%	n/a (already total)
70%	73 → **70%**	76 → **80%**	**Yes** in both (one disability ≥60%)
50%	55 → **60%**	60 → **60%**	No in both (combined under 70%)
30%	37 → **40%**	43 → **40%**	No
Denied	**10%**	19 → **20%**	No

The ankle changes the monthly amount at 70%: $1,961.45 at 70% against $2,277.15 at 80%, for a veteran with a spouse at 12/1/2025 rates (these change with the 12/1/2026 COLA) (`PATH-TO-100-WITHOUT-TDIU` App. B). It doesn't change whether 4.16(a) is met at any mental-health level.

Combined 100% from 70% needs, in addition, sleep apnea at 50% (CPAP required) **plus three** more genuine 30% conditions (70 + 50 + 30 + 30 + 30 → 85 → 90 → 93 → 95, which rounds to 100), or a second 50% condition plus one 30%. With only two 30% conditions the combined value is 93, or 94 with tinnitus, and that rounds to **90%** (38 CFR 4.25, Table I). One more genuine 10% condition would tip it: for example, the ankle if it is service-connected at 10%. In that case 70 + 50 + 30 + 30 + tinnitus 10 + ankle 10 → 85 → 90 → 93 → 94 → 95, which rounds to **100%**. So the realistic minimum is sleep apnea at 50%, two 30% conditions, tinnitus and one more 10%. `PATH-TO-100-WITHOUT-TDIU` Route 2 says two 30% conditions are enough; that is an arithmetic error. VA has also proposed removing the CPAP-based 50% for sleep apnea (proposed rules 2/15/2022 and 9/12/2024, not final as of 10/13). Claims pending when a final rule takes effect generally get whichever criteria are more favorable. Today:

- there is no sleep study and no diagnosis (consult only as of 7/31);
- there are no 30% conditions;
- hypertension (lisinopril since 2021) would add about one point.

Fit: poor. Sleep apnea is worth pursuing on its own, since it takes 70% to 90%. File the Intent to File if it isn't already filed; filing status is unconfirmed. Only genuine conditions should be pursued.

6.2 TDIU, 38 CFR 4.16(a)

Requirements (verified 2026-10-13 against 38 CFR 4.16 via Cornell LII):

- Unable to secure or follow a substantially gainful occupation because of service-connected disabilities.
- One disability ratable at 60% or more, or, with two or more, one at 40% or more and a combined rating of 70% or more.
- "Marginal employment shall not be considered substantially gainful employment." Marginal employment generally exists when earned annual income doesn't exceed the Census poverty threshold for one person.
- *Faust v. West*, 13 Vet. App. 342 (2000), treats income above that threshold as substantially gainful regardless of hours worked. It defined that in the context of reducing a total rating (38 CFR 3.343(c)), and the Board applies the same definition to TDIU. It isn't absolute: under 4.16(a), above-threshold income can still be "marginal employment" "on a facts found basis," including "employment in a protected environment

such as a family business or sheltered workshop." The same paragraph says "consideration shall be given in all claims to the nature of the employment and the reason for termination." That bears directly on the family CPA-firm plan.

- TDIU is part of the initial rating when the record reasonably raises it (*Rice v. Shinseki*, 22 Vet. App. 447 (2009); verified in the corpus 10/8, `REASSESSMENT` evening update; `vre/` lists it as unverified). My 10/7 21-4138 already raises it.

Fit:

- **For:** unemployed since 6/2026; an involuntary termination after FMLA leave; the Metal attendance record; treatment interruptions at every documented job.
- **Against:**
  - two layoffs;
  - high-wage skills and current offers;
  - consulting plans;
  - possible unemployment certifications and VR&E statements of capacity;
  - SUD attribution, since TDIU counts only service-connected disability.

What's missing:

- service connection at 70%;
- a 21-8940 (nothing filed as of 9/14; `vre/ACTION-CARD.md`);
- employer 21-4192s;
- the MNTN HR, PIP and FMLA records;
- short-term disability files with attending-provider statements;
- a settled, consistent account of work capacity.

Ask James whether to file the 21-8940 now or right after a grant, and check the VR&E earned-income interaction first (`REASSESSMENT` evening update; `vre/OPTIONS-REPORT-2026-09-14.md` §7).

6.3 Extraschedular TDIU, 38 CFR 4.16(b)

Rating boards "should submit to the Director, Compensation Service, for extra-schedular consideration all cases of veterans who are unemployable by reason of service-connected disabilities, but who fail to meet the percentage standards" of (a) (38 CFR 4.16(b), verified 10/13). It matters only if mental health is rated 50% or lower. With tinnitus (and maybe the ankle) as the other conditions, unemployability would have to rest almost entirely on the mental-health condition at a level VA has already found below 70%. **Fit: weak.** The record is silent on any 4.16(b) analysis.

6.4 Temporary total for hospitalization, 38 CFR 4.29

Requirement (verified 10/13): a service-connected disability "has required hospital treatment in a Department of Veterans Affairs or an approved hospital for a period in excess of 21 days," or VA-paid hospital observation over 21 days. Under 4.29(b), if service connection is granted after admission, the rating runs from the first day of hospitalization "if otherwise in order." On residential care, one Board decision holds that a residential/PHP step-down "is not eligible for a TTR" because it was not hospitalization. The same decision requires "continuous hospitalization in excess of 21 days" and declined to qualify separate 10- and 13-day admissions (BVA A25017339, 2/26/2025; non-precedential). The corpus also cites M21-1 VIII.iv.8.B for the residential point; that manual text couldn't be verified (`PATH-TO-100-WITHOUT-TDIU` Route 3).

Fit:

- **August overdoses:** each lasted less than a day. The 8/4 admission was at 06:04 with discharge at 6:35 PM, and 8/5 was an ED visit. They are nowhere near 21 days and **don't qualify.**
- **Harmony, 4/17–5/11/2026 (about 24 days):** it was "Detox, Residential," at a private facility, with SUD diagnoses only on the discharge summary, and it predates the likely 5/19/2026 effective date. The medically monitored detox phase was well under 21 days, and the Board applies a continuous-hospitalization requirement (A25017339). **Very unlikely.** It becomes a live question only if an earlier effective date is established, and even then the type of care is doubtful.
- **Future stays:** only a genuine stay over 21 days of hospital-level care after service connection. Treatment length is a clinical decision.

6.5 Surgery or cast, 38 CFR 4.30

Closed. It requires surgery or cast immobilization (`PATH-TO-100-WITHOUT-TDIU` Route 4).

6.6 The effective-date lever (not a percentage)

If service department records (STRs or OMPF) that existed at the 2024 decision are newly associated and support the award, 38 CFR 3.156(c) requires reconsideration. An award based on those records is effective from the date VA received the 2024 claim or the date entitlement arose, whichever is later (3.156(c)(3)). It doesn't apply to records VA couldn't get because I didn't give enough information to find them. Otherwise 3.2500(h)(2) likely caps it at receipt, 5/19/2026, if that filing was a supplemental claim made more than a year after the 2024 decision notice. The corpus estimates about $35–47K at 70% (arithmetic only). The 2024 decision date isn't in the files (`PATH-TO-100-WITHOUT-TDIU` §3).

7. Honest odds

All figures are reasoned estimates, not statistics. They assume the exam has not yet produced a DBQ that changes the picture.

The latest dated estimates in the corpus (both 10/8, and they differ):

- Service connection: about 55–65% (`REASSESSMENT-2026-10-08-CP-EXAM.md` evening re-score).
- Schedular 100%, `REASSESSMENT` revised table (late 10/8): about 10–15% if service connection is granted, and about 6–9% unconditional. The late-10/8 overdose update gives about 10–14% and 6–9%.
- Schedular 100%, `PATH-TO-100-WITHOUT-TDIU-2026-10-08.md` "Corrections after the consistency check": about 12–18% if service connection is granted. Unconditional, about 7–11% at 12 months and about 13–20% at 36 months.
- Total compensation by either route, unconditional: about 20–30%, with TDIU at about 40–55% given a 70% grant (`REASSESSMENT` revised table and re-score).
- A 10/8 second-pass note puts the odds "on hold until after the exam," because they rested on hospital records I say are inaccurate (`CP-EXAM-PREP-MENTAL-HEALTH.md`, overdoses).

My estimates:

Outcome	REASSESSMENT (10/8)	PATH (10/8)	This report
Service connection granted	~55–65%	~55–65% (in its math)	~55–65%
Schedular 100%, if service connection is granted	~10–15%	~12–18%	**~8–15%**
Schedular 100%, unconditional	~6–9%	~7–11% at 12 months	**~5–9%** (initial decision)
TDIU, if granted at 70%	~40–55%	not estimated	**~35–50%**
Paid at 100% by either route, unconditional	~20–30%	not estimated	**~15–25%**

The most likely single outcome in every source is a grant at 70%.

Where I differ and why:

1. **Schedular 100% is slightly lower than PATH and at the low end of REASSESSMENT.** PATH moved from about 10–14% to about 12–18% in the same correction that downgraded Cherie's "high risk" line to a conditional template. That is evidence getting weaker while the estimate went up. The rise seems to come from the family-contact correction, which is real but modest. Nothing since 10/8 adds a clinician finding at the 100% level. The filed overdose 21-4138 adds my account of SI at the time, but a later self-report against the records made that day tends to be weighed as a credibility question rather than as proof of persistent danger. I keep the "on hold" caveat: an examiner who documents persistent danger, or total impairment on the evidence, would move this up substantially.
2. **TDIU is slightly lower.** The 10/8 TDIU estimate doesn't weigh the `vre/` employability facts: recruiter offers, the consulting plan, possible weekly unemployment certifications, and an unknown VR&E finding. Each could be a VA-visible statement of capacity. If none of them exists in a VA-visible form, or the true picture is that I can't sustain work, the corpus range is reasonable.
3. **The arithmetic behind the bottom row.** About $0.6 \times (0.11 + 0.6 \times 0.42) \approx 0.22$, so about 15–25%. TDIU after a 70% grant remains the more probable path to the 100% rate, as the corpus says.
4. **The 10/13 corrections didn't move these numbers.** The consistency check narrowed or corrected several facts:

- Cherie's "unable… takes over" self-care sentence is now weighed;
- Erica's account includes things she does for me, not just reminders;
- the 8/4 and 8/5 records are treated separately;
- the June relapse is added.

They move the self-care verdict from "doesn't meet" to "approaches." They don't change the estimates. Cherie's sentence and Erica's statement were already in the record the 10/8 estimates were built on, neither gives a frequency, and the Board sample treats family-reported ADL help without examiner documentation as insufficient. The 8/5 kratom entry supports my account of what I used, not persistent danger.

What would move these:

- **Up:** a DBQ marking total impairment or persistent danger; post-August treating records showing an ongoing course; an independent evaluation with valid testing; employer and short-term disability records tying the job losses to symptoms.
- **Down:** a DBQ finding SUD primary; an invalid testing profile; VR&E or unemployment records stating capacity; the August events read as isolated and followed by stabilization.

8. Records and evidence that would change the picture

Ranked by impact on a total rating.

#	Record or evidence	What it settles	Who holds it
1	**C&P DBQ** (if the exam happened), via VA Form 20-10206 or James in VBMS	The examiner's impairment box, the SUD attribution, the marker opinion, validity	VA / VES; James
2	**Confirmation of what VA holds:** the signed overdose 21-4138 and its final text; both 10/7 forms; the 10/9 kratom correction and its fix; which nexus letter(s)	Whether my corrections are actually in the record	James (VBMS)
3	**Cherie's complete chart, 2/2023 to present, including any post-August notes**	Course over time: persistent or episodic SI, hygiene, attendance	Cherie Mills (request or 21-4142)
4	**Treating-provider Mental Disorders DBQ (21-0960P-2) or an independent psychological evaluation with validity testing**, each in the clinician's own judgment	Whether a qualified examiner finds total impairment; addresses the denials and disclosure timeline	Cherie; an independent psychologist or psychiatrist (`IMO-PROVIDER-RESEARCH-2026-10-12.md`)
5	**Employment records:** MNTN leave approval, PIP/HR emails, termination; short-term disability files with attending-provider statements; Meta/DISQO leave records; DISQO separation; 21-4192s	Whether job losses track symptoms; TDIU	Former employers; leave administrators and STD insurers
6	**VR&E file:** 9/17 VRC notes, 28-1902w, any entitlement finding	What the C-file already says about my capacity to work	VRC Jamie Clark / VR&E

#	Record or evidence	What it settles	Who holds it
7	**Texas unemployment records, if a claim was filed**	Whether I've certified ability to work	Texas Workforce Commission / me
8	**Post-August medication management and treatment** (Reichmann/SDKare; any program since)	Current course and current use	SDKare; providers
9	**STRs and OMPF**	Service connection markers; soundness; the 3.156(c) effective date	VA Records Management Center; NPRC (SF-180)
10	**Dr. Bowers's records, 2020–2024**	Years of psychiatric course; the probable OCD diagnosis	Bowers's office
11	**Full August hospital set, including the tele-psych consult note**	Exactly what was recorded about intent	South Austin Medical Center; the 8/5 ED
12	**Starlite and Victory complete records** (facility 21-4142s were never filed)	The ASAM "suicide attempt" checkbox; trauma entries; pre-service entries	Starlite; Victory
13	**Sleep study**	The sleep apnea secondary (70 → 90 combined)	Sleep clinic

Each record matters for what it shows, whichever way that cuts. VA may obtain any of them anyway (`RECORDS-TO-GET-AND-PATHS-TO-100-2026-10-08.md`, "How to read this").

9. Unresolved

These are for me or James. Where the rules don't settle a conflict, both sides are cited and I don't pick one.

1. **Exam status.** Scheduled for 10/13 at 12:00 PM with Dr. Howard (`CP-EXAM-PREP-2026-10-13.md` §2) against "rescheduled by VA; date TBD" (`CP-EXAM-NOTES-2026-10-13-SOURCES.md` §7.1).
2. **Ankle service connection.** "Service connected at 10%… effective May 19, 2026" (personal statement §6) against tinnitus as the only service-connected condition (`PATH-TO-100-WITHOUT-TDIU` Route 2). Changes 70% to 80% (§6.1).
3. **The overdose 21-4138 and the 8/4 medications.** The filed text (`personal-records/21-4138-AUGUST-2026-OVERDOSES-FILED.md`) explains the minimizing, but not the other medications the 8/4 record lists or the positive benzodiazepine screen. Did I take any of my prescribed medications that night, as well as 7-OH? 3a. **A 10-day residential stay after August.** The filed 21-4138 says I "recently spent 10 days in a residential inpatient treatment center" and stopped using "for a short time." Per my account (10/13): Victory Addiction Recovery Center, 8/8–8/18/2026. The discharge records aren't in the file yet. Get them directly or through a 21-4142/4142a.
4. **The persecutory belief.** "A fixed, certain belief… plans to kill him… nuclear bunker" (`CORPUS-AUDIT-REMEDIATION-PLAN-2026-07-06.md` B15) against a recurring worry (settled 10/8–10/13). The current version controls. When it began, after service or about 18 years ago, is still open (`CP-EXAM-PREP-2026-10-13.md` §9 item 13).

5. **ADLs.** Three accounts with no frequency: "feeds him, cuts his hair" (`RATING-REPORT-REMEDIATION-PLAN` [CORRECTION 3]); prompting (veteran's account, 10/8); and Erica's mix of reminding and doing (appointments, clothes, grooming, groceries, sleep medications). Cherie's "during especially bad stretches… his wife takes over" sits against Erica's "constantly remind" during "the worst bouts."
6. **Cherie's letter beyond my 10/8–10/13 account.** "Estranged… 6 to 7 years," "picks out the clothes," "paranoid delusions" and "FMLA… at every job he has had since I have worked with him" (7/17 letter) against my settled account. Metal, with sick leave and no FMLA, is the only job that conflicts with the FMLA statement. Only Cherie can revise her letter, and it would be her decision.
7. **Marriage.** My signed statement has both "healthy and supportive" and "strained by the symptoms above" (personal statement §8), against "total impairment… caregiving dependency" (Cherie) and "roommates at best and enemies at worst" (Erica). Only I can say whether "healthy and supportive" was accurate when I wrote it.
8. **SI frequency.** Daily (personal statement; Cherie) against seven denials in April–May, and my own account that mid-May was SI-free. Whether the hold-avoidance explanation fits the April entries is mine to decide (`CP-EXAM-PREP-MENTAL-HEALTH.md`).
9. **Pre-service use.** "Infrequently… not a habit" (10/9 correction) against "regular use 18–20" for opiate pills (Victory p. 8). Victory's "cocaine at 19" is addressed by the 10/9 correction ("I do not recall using cocaine before service"). The age-18 DUI (later expunged, per intake self-report) and the age-20 paraphernalia arrest (OCR) aren't addressed in any filing.
10. **Sobriety in July 2026.** "Currently sober" (Cherie's progress notes, 7/9 and 7/16) against the early-June 2026 relapse (opioid pills and kratom, Suboxone stopped; `COMPREHENSIVE-FINDINGS-2026-07.md` §2; `vre/VRE-EVALUATION-PREP-2026-09-17.md` §3.1 J2) and heavy 7-OH before August. Current use is undated and unquantified ("my struggles with it continue to this day," 10/9 correction).
11. **Drug spending.** More than $30,000 on 7-OH over two years, and about $500 a day (veteran's account, 10/8), against "around $30,000 on drugs over the course of our marriage" (Erica). The first two are mine to reconcile. Erica's figure is hers.
12. **Mountain.** End date 6/26 (agreement) against 6/30 (10/7 Part 2). Reason: none stated (agreement), "related to my disabilities" (Part 2), and a performance/PIP loss (remediation plan).
13. **The Dec 2024 job loss.** Condition-caused (`CLAIM-RECONCILIATION-WORKLIST.md` B1) against "headcount reduction for financial reasons" (Metal statement).
14. **Employability statements.** Whether a Texas unemployment claim was filed. If one was filed, its weekly able-and-available certifications are obtainable, and the fact has to be stated consistently in every VA-visible file. Also open: what I said at VR&E on 9/17, and the VRC's finding (`vre/FINANCIAL-BRIDGE-2026-09.md` §1.8; `VRE-EVALUATION-PREP` §8.6).
15. **Residence and exam location.** The 1300 E Riverside / 1700 Clifford Ave addresses in Austin (VES letter; 10/7 forms) against a relocation to Lake Charles around 9/2026 (`vre/README.md`; `vre/VRE-EVALUATION-PREP-2026-09-17.md` §3.1 row K, "inferred"; `vre/FINANCIAL-BRIDGE-2026-09.md` header). This bears on examiner licensing and notice.
16. **Which nexus letter(s) VA holds** (7/2, 7/17 or both), and whether Cherie approved the final text and image signature (`REASSESSMENT` §5).

17. **The 2024 decision:** its date and what was claimed. That decides the one-year rule and the 3.156(c) question.
18. **Cherie's materials-reviewed list.** Her letter says she reviewed STRs and personnel records. The corpus says the STRs aren't in hand (`RECORDS-TO-GET` #2).
19. **Nick White's lay statement (21-10210, corrected version dated 10/13/2026; `supporting-docs/VBA-21-10210-ARE-Nick-Complete-flattened.pdf`).** New evidence, added after this report was first drafted. It gives a pre-service baseline ("outgoing," social, in college with me 2007–2009) and acknowledges a short pre-service "period of depression" with possible therapy, which matches my own 10/7 account (psychiatrist ~19). It dates my disclosure to him to "toward the end of 2013 after he got out," which predates the 2015 VA denial. From 2015, as roommates during the McNeese years, it describes post-service change: isolation, declined invitations, disheveled, belongings disappearing, drug use framed as escape from "depression, anxiety, panic, and not being able to sleep." Those are 3.304(f)(5)-type markers. An earlier version said I was "currently on unemployment"; the corrected one doesn't. **Open:** the corrected version's signature was carried over from the original, and a re-signed copy isn't on file yet; only one version should reach VA. His pre-service picture still doesn't mention the ages 18–20 substance history in the records. "Home visiting family" bears on residence and family contact. Only Nick can change his own wording.

10. Sources

Primary records (cited by filename; PHI quoted minimally):

- `supporting-docs/Marcus Klein Cherie Mills Nexus Letter -2026 (3).pdf` (7/17/2026)
- `supporting-docs/EricaStatementSigned (2).pdf` (7/12/2026)
- `supporting-docs/VA Form 21-4138 Part 1 Official (1).pdf` and `Part 2 Official (1).pdf` (10/7/2026)
- `supporting-docs/VA_21-4138_Substance_Use_Correction_Flattened.pdf` (10/9/2026, OCR)
- `supporting-docs/VBA-21-0781-Signed-Personal-Statement-Finalized (1) (1).docx` (6/17/2026)
- `supporting-docs/MetalManagerEmploymentStatement.pdf` (8/2/2026); `MetalSeparationDocs.pdf`; `eb99f8e9-…-signed.pdf` (Meta)
- `supporting-docs/08042026_Overdose_Discharge.pdf`, `_Physical.pdf`, `_Provider_Report .pdf`, `_Summary.html`; `08052026_Overdose_ER.pdf`
- `supporting-docs/marcus DC summary (1).pdf` (Harmony, 5/11/2026); Naficy notes (`marcus 3 (1).pdf` and others); `Cherie CPT Progress Note.pdf` (7/16) and Notes 2–5
- `supporting-docs/Marcus-WHODAS_2_0-11Jun2026 (1).pdf`; `Psych-DASS21-09Jun2026.pdf`; `Victorry2018Records (1).pdf`; `PocketBuddyLetter.md`; `VA-Buddy-Form_reyes_complete (1).pdf`
- `personal-records/MNTN-Separation-Agreement-2026.pdf`; `personal-records/21-4138-AUGUST-2026-OVERDOSES-DRAFT.md`; `personal-records/CP-EXAM-STRATEGY-2026-10-13.docx` (via the Slice B extract)

Corpus analysis (tier 3 unless noted):

- `planning-docs/CP-EXAM-PREP-2026-10-13.md` §1–§9
- `planning-docs/CP-EXAM-NOTES-2026-10-13-SOURCES.md` §7.1

- `planning-docs/REASSESSMENT-2026-10-08-CP-EXAM.md`, including the evening and late updates
- `planning-docs/PATH-TO-100-WITHOUT-TDIU-2026-10-08.md`
- `planning-docs/RECORDS-TO-GET-AND-PATHS-TO-100-2026-10-08.md`
- `planning-docs/CP-EXAM-PREP-MENTAL-HEALTH.md` (superseded; used for the 10/8 facts)
- `planning-docs/RATING-REPORT-REMEDIATION-PLAN-2026-07-16.md`
- `planning-docs/CLAIM-RECONCILIATION-WORKLIST.md`
- `planning-docs/CORPUS-AUDIT-REMEDIATION-PLAN-2026-07-06.md` and `PATH-TO-100-PERCENT-2026-07-07.md` (history only)
- `CLAIM-ASSESSMENT.md` Part 0
- `vre/OPTIONS-REPORT-2026-09-14.md`, `VRE-EVALUATION-PREP-2026-09-17.md`, `FINANCIAL-BRIDGE-2026-09.md`, `ACTION-CARD.md`, `README.md`
- `IMO-PROVIDER-RESEARCH-2026-10-12.md`
- `rater-kb/` (all six files)

Not reused:

- the retracted ~6/25/2026 SI denial;
- every "90% mental health" route;
- the delusion and "feeds him" versions;
- the claim-maximization coaching and staged-release advice;
- `OPTIONS-REPORT` §7.7 #3 on not telling an examiner about a business.

External authorities:

- 38 CFR 4.130; 4.126(a)–(b); 4.16(a)–(b); 4.29(a)–(b); 4.25; 4.30; 3.156(c); 3.2500(h)(2); 3.301(c)(3); 3.102
- *Mauerhan v. Principi*, 16 Vet. App. 436 (2002)
- *Vazquez-Claudio v. Shinseki*, 713 F.3d 112 (Fed. Cir. 2013)
- *Bankhead v. Shulkin*, 29 Vet. App. 10 (2017)
- *Allen v. Principi*, 237 F.3d 1368 (Fed. Cir. 2001)
- *Mittleider v. West*, 11 Vet. App. 181, 182 (1998)
- *Faust v. West*, 13 Vet. App. 342 (2000)
- *Rice v. Shinseki*, 22 Vet. App. 447 (2009)
- *AZ & AY v. Shinseki*, 731 F.3d 1303 (Fed. Cir. 2013)
- M21-1 VIII.iv.8.B (as read in the corpus; unverifiable 10/13)
- BVA A25017339 (2/26/2025)
- 38 CFR 3.343(c); 4.97 DC 6847

The 4.16, 4.29 and 4.126 texts were re-checked on 10/13 against Cornell LII. A citation-verifier pass on 10/13 checked every citation in this report against Cornell LII, govinfo, va.gov, the Federal Register and the opinions or Board decisions quoting them. It corrected the combined-rating route (§6.1), the 4.16(b) wording, the *Faust*

scope, the 3.156(c) effective date and the 4.29 Board support. The M21-1 text and *Bankhead*'s exact reporter page remain unverified.

*If things get dark: call **988 and press 1** (Veterans Crisis Line), or text **838255**.*